



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : فاطمه ابراهيم محمود رزق
المحافظة : الجيزة
الحى / المدينة : ابوالنمرس
رقم البطاقة : 25604072100562 رقم التليفون : 01028966037
العمر (تاريخ الميلاد) : 07-04-1956 المهنة : ربه منزل
العنوان : -
تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (.....)
المقر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان القولون
نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان القولون
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [6161] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة:
الوظيفة:
الوظيفة:
التوقيع:
التوقيع:
التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: فاطمة ابراهيم محمود رزق
National ID: 25604072100562



تقرير طبي

المسجد

تحية طيبة وبعد,,

بشان تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

A patient of colon cancer for Further investigations and imaging

الوقت :

-- --

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
 وزارة الداخلية
 فبطمة
 ابراهيم محمود رزقي زينتون
 قرية نزلة الاشطر
 مركز أبو التمرس - الجيزة
 ٢٠٢٦.٠٤.٠٧ ٢١.٠٠.٥ ٦٢
 LT5944396



٢٠٢٦/٠٥ ٢٥٦.٤.٧٢١.٠٠٦٢
 ربة منزل
 لثني مسلمة متروجة
 عزازي سعودي عامر صبيح
 البطاقة سارية حتى ٢٠٢٦/٠٥/١٤



05/03/2026

MDT FORM

Name:	Fatma Ibrahim Mohamed *	MARN:	1306
Age:	70 Y	ID:	25604072100162
Menopausal Sta	post	Phone:	01097000921
NRS SCORE:		Marital Status:	married
BMI:		Family History:	
CO Morbidity:	DM, HTN	Reason Of MDT:	

Clinical Data:

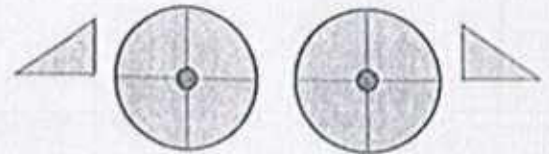
BX:	10C GII	R	L
ER:	8/8	PR:	8/8
HER2:	+	KI67:	10

Sonomammography:

Mets Workup:

History:

double lry breast and colon
 hx of LT breast cancer underwent LT SM + SLNB in 2/2024 > T2N0, received
 PORT and kept on hormonal ttt since that
 last assessment by ct cap >> showing bilateral pulmonary nodules largest
 11mm and 9mm, biopsied >> mets colorectal adenocarcinoma (21/12/2025)
 colonoscopy done 22/1/2026 >> cauliflower mass lesion at 15cm from anal
 verge with bx showing adenocarcinoma GII
 N.B: PS 2-3



TNM

Recommendations:

MDT Outcome & Plan

For new CT-Chest abdomen pelvis to be tested for K/RAS meanwhile start
 Xelox

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنا بركات					

Name: فاطمة ابراهيم محمود		
MR. : 1306		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه The patient has a history of double primary malignancies, including left-sided breast cancer treated with left simple mastectomy and sentinel lymph node biopsy in February 2024 (T2N0, ER 8/8, PR 8/8, HER2-negative, Ki-67 10%). Post-operative radiation therapy was administered to the left breast and regional nodes, and the patient has remained on hormonal therapy (tamoxifen) since that assessment. In December 2025, a CT scan revealed bilateral pulmonary nodules (largest 11 mm) and hilar/femoral lymphadenopathy; a biopsy confirmed metastatic colorectal adenocarcinoma. A colonoscopy performed in January 2026 identified a cauliflower-shaped mass 15 cm from the anal verge, with biopsy demonstrating grade II adenocarcinoma (PS 1-2). Multidisciplinary team discussion recommended a new CT of chest, abdomen, and pelvis to evaluate for KRAS mutation status, and initiation of Xelox chemotherapy. On 19 May 2026, the oncology team approved initiation of Xelox therapy, with concurrent hormonal therapy (Femara) and supportive measures. By 20 June 2026, the patient's performance status was documented as 2; Xelox was started with a dose reduction and nutritional support was instituted. On 15 August 2026, the patient (weight 45 kg, height 165 cm, BSA 1.5 m²) received the first Xelox dose of oxaliplatin 185 mg IV every 3 weeks, combined with oral capecitabine (3 days on, 2 days off) for a 14-day cycle. The Femara schedule was noted to be due on 27 August 2026. On 6 September 2026, the oncology team documented the patient's metastatic disease burden, including liver metastases (segment 8, 3 × 2.7 cm; segment 5, 13 × 8 mm) and ground-glass inflammatory lesions. KRAS, NRAS, and BRAF testing returned wild-type. The patient had completed one cycle of Xelox. The multidisciplinary team discussed adding targeted therapy (potentially FOLFOX) and planned a second cycle of Xelox with oxaliplatin 125 mg and capecitabine 3 days on, 2 days off for 14 days. The patient has completed 2 cycles of Xelox to date.		
تحريراً في ,		
	الوقت :	التاريخ : 19-09-2026
هنا أسامه عبد الرحيم عبد البر		الطبيب المعالج :
يعتمد ,		

Patient's Name: Fatima Ibrahim Mahmoud
Date: 2026-06-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported left breast carcinoma .

(I) Chest:

- An area of ground glass attenuation with bubbly lucencies is seen within right upper lobe anterior segment ,another similar lesion is seen within the lingula .
- Mosaic perfusion of both lung lobes .
- No sizable hilar or mediastinal lymph nodes .
- No sizable pulmonary nodules .
- Clear both lung fields .no sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density , segment VIII focal lesion measures 3 x 2.7 cm , another lesion seen at segment V measures 13 x 8mm.
- Patent and homogeneously enhancing portal vein.

Patient's Name: Fatima Ibrahim Mahmoud
Date: 2026-06-01
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys , aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.

Skeletal system :

- Spondylodegenerative changes .
- Mottled appearance of bone with multiple lytic bony lesions ..
- No vertebral structural collapse .

OPINION:.

- Multiple area of ground glass attenuation with bubbly lucencies ?? neoplastic ?? infectious for clinical correlation .
- Two right lobe hepatic focal lesion likely metastatic
- Mottled appearance of bone with multiple lytic bony lesions for bone scan correlation .

Dr.Reham Khedr, MD

Dr.Rahma Khader

Dr.Rahma Khader

Name : FatmaIbrahim

Date : 06-07-2026 06:35

Branch : Dokki

Medical No. : 409873

Ref.Physician Prof : Goustav Roussey Egypt

Whole-Body Bone Scintigraphy

Indication: Follow-up of a known primary left breast carcinoma, status post-left mastectomy in 2022. Investigation for potential skeletal metastasis in a patient presenting with generalized bone aches.

Procedure: Whole-body planar bone scintigraphy performed in anterior and posterior projections approximately 2 to 3 hours following the intravenous administration of Tc-99m MDP.

Findings

Axial & Appendicular Skeleton: Normal, symmetric tracer distribution throughout the skeleton. No focal areas of intense radiotracer hyperfixation or photopenic defects are identified to suggest osteoblastic or osteolytic metastatic disease.

Joints: Mild, symmetrically increased tracer localization is noted at the ankles/tarsal regions bilaterally, consistent with benign, age-related degenerative or mechanical changes.

Soft Tissue & Excretion: Expected soft-tissue background clearance with physiological tracer excretion in both kidneys and the urinary bladder.

Impression

- No scintigraphic evidence of active osteoblastic osseous metastasis.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

Patient Name	فاطمه ابراهيم محمود رزق زيتون	Case No.	264380
Sex/Age	Female 70 Y	Patient No.	20321
Registered Date	03/6/2026 12:59:18 PM	Referred By Dr.	Out Patient
Validation Date	25/6/2026 02:15:10 PM	Branch	Gustave Roussy
Reporting Date	28/7/2026 12:06:42 PM	Pathology No.	657/26

Pathology Report

Clinical Data: Cauliflower mass at 15 cm from anal verge in a patient with history of breast cancer.

Nature of specimen: Referred slides (Colonoscopic biopsy).

Gross Examination:

Five referred unstained slides were received, labelled by patient name.

Microscopic Examination:

Histopathological examination reveals fragments of colonic mucosa showing infiltration by tumor formed of glands and cribriform pattern lined by malignant columnar cells showing enlarged moderately pleomorphic nuclei amid desmoplastic stroma. There is focal necrosis.

Diagnosis:

Mass at 15 cm from anal verge, Referred slides (Colonoscopic biopsy):

- Invasive adenocarcinoma, Grade 2.

Signature

Dr. Walaa Ibrahim Ghoneim.



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:
1- Name: **د. نوره إبراهيم محمود زلق**

Sex:

MDT form

3- Medical numbers:
Female

63164315

Old Med:

DATE:

16-2024

2-ID numbers: 25604072100562

Patient Age: 68 Y

Family history: Family History: +ve
marital status: Marital Status: Married
Co morbidity: Co morbidity: dm, htn
Phone: 0111018816
NRS Score: NRS Score
Initial STAGE: Initial STAGE
Date of MDT: 16-2024

Breast Imaging:

Mets Workup:

ct cap: bilateral pulmonary nodules for follow up
bilateral iliac osseous lytic lesions
been scan: +ve solitary lumbar lesion for IV3
radiological degenerative metastatic

BX IDC EX COMMENT: GI	ER 8/8 EX COMMENT:	PR 8/8 EX COMMENT:	Her 2 Neu 1 EX COMMENT:	SISH EX COMMENT:	167 10
T 2	N 0	M 0			

Sonomammography LT UOQ BIRADS 4a

PATIENT HISTORY

RT BREAST CYTOLOGY FREE FROM MALIGNANCY

MRI LUMBUSACRAL: NO METASTASIS

LT SM+SLNB: IDC I+major DCIS

close margin 1mm

free skin, LN 0/5

POST-OPERATIVE PLAN

- 1- PORT
- 2- ADJUVANT ADJUVANT AI
- 3- ZOMETA

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name: Fatma Ibrahim Mahmoud Rezk Zaytoun	Date : 11/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 68 Year(s)
Medical ID: 3587/2023	Pat. ID: 42106

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is heterogeneously dense which may obscure small lesions (ACR C),
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions
- Free left infra and supra clavicular regions.

Opinion:

- Left MRM with free operative bed with no evidence of recurrent mass lesions
- No right breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Walaa Abdelazez
11/12/2024 12:00 PM

Patient Name : الأستاذة / فاطمة إبراهيم محمود	Request No. : 13002984
Patient No. : 25604072100562	Request Date : 31/08/2023
Path No. : 2929/23	Verification Date : 10/09/2023
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 67 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Left breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 31/8/2023, 10:00 am.

Gross Examination:

- Four grayish white tissue cores up to 1 cm, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Extensive DCIS.
- * Perineural invasion: No.
- * Angioinvasion: No.

Diagnosis:

Left breast mass, Tru-cut biopsy:

Invasive duct carcinoma G II with extensive DCIS.

For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :

Patient Name: Fatma Ibrahim Mahmoud Rezk Zaytoun	Date : 18/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 69 Year(s)
Medical ID: 3587/2023	Pat. ID: 42106

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to the previous study dated 20/9/2023, the current study showed:

1. Chest:
 - Status post left mastectomy.
 - Bilateral apical reticular opacities with right sided honeycombing.
 - Multiple newly developed bilateral lower lobe pulmonary nodules largest measuring 11 mm at the left lower lobe.
 - Stationary course since last study regarding the size of the rest of the previously seen bilateral pulmonary nodules.
 - Newly developed left upper lobe subpleural reticulations and ground glass density ? post radiotherapy sequelae.
 - Mild progressive course regarding the previously seen pretracheal lymph node measuring 14 mm in today's study compared to 12 mm in the previous study
 - No evidence of pleural effusion, pleural thickening or masses seen.
 - Average cardiac size. No pericardial effusion detected.
 - Normal CT appearance of the thoracic aorta and the great vessels.
2. Abdomen and pelvis:
 - The liver moderately enlarged with homogenous CT texture.
 - Newly developed segment IVa well defined hypodense focal lesion with peripheral contrast uptake measuring 12 x 15 mm likely metastatic.
 - Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
 - No intra-hepatic or extra-hepatic biliary radicles dilatation.
 - Normal size & parenchymal texture of the spleen with no focal lesions seen.
 - No pathological abdominal or pelvic lymph nodes.
 - Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
 - Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
 - Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
 - Normal appearance of the other pelvic organs.
 - No abdominal or pelvic masses or collections.
3. The scanned skeleton:
 - Bilateral iliac wings lytic lesions are still noted.
 - Heterogenous CT attenuation of the examined vertebra ? osteopenic for bone scan.

Dr. Sherif Wagdy

Dr. Sherif Wagdy

Patient Name : الأستاذة / فاطمة ابراهيم محمود رزق
Patient No. : 25604072100562
Path No. : 3355/25
Sex : Female
Age : 69 Y
Referring Doc :

Request No. : 13000961
Request Date : 03/11/2025
Verification Date : 30/11/2025
Financial Category : جوماتاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right hepatic lobe focal lesion, in known case of breast cancer.
Nature of Specimen: Right hepatic lobe focal lesion, Tru-cut biopsy, collected on 02/11/2025.

Gross Examination:

- Two grey white tissue cores, each measured 2 cm long. Totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; GATA3 clone (L50-823), CK7 clone (SP52), CK20 (SP33) & CDX-2 clone (EPR2764Y).

Microscopic Examination:

- * Histopathological examination reveals cores of hepatic tissue showing infiltration by tumor formed of malignant epithelial cells showing moderate nuclear pleomorphism and eosinophilic cytoplasm, arranged in glands with fused forms among desmoplastic stroma. There is focal necrosis.

- * IPT (Positive control showed appropriate reactivity):

- GATA3: Negative in the tumor cells.
- CK7: Negative in tumor cells.
- CK20: Positive in tumor cells.
- CDx2: Positive in tumor cells.

Diagnosis:

- Right hepatic lobe focal lesion, Tru-cut biopsy:
Metastatic adenocarcinomatous deposits (Positive CK20 & CDx2), mostly consistent with colo-rectal / GIT origin.
- Case of clinical & radiological correlation.

Signature

Prof. Dr. Maha Guimei.

Verified by :

Dr. Duaa Mohamed

Printing Date : 30/11/2025 10:21:36

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Room No. (4) - First Floor-Building No. 987- Nile Corniche - Missr El Qadema - Cairo, Egypt
جودة رقم (4) - الطابق الأول - مبنى رقم 987 - نيل كورنيش - مصر القديمة - القاهرة، مصر

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Page 2 of 2

Printing Date : 30/11/2025 10:21:36

Verified by :
Dr. Duaa Mohamed

Dr. Walaa Ibrahim Ghoneim

Pathology Report

Refering Doc :	Registered by
Age	Department
Sex	Financial Category
Path No.	Verification Date
Patient No.	Request Date
25604072100562	Request No.
3355/25	13000961
Female	
69 Y	
Out Patient	
Mrs. Mayada Hegazy	

ISTAVE/
JUSSY/
NATIONAL

Patient Name : الأستاذة / فاطمة إبراهيم محمود رزق
Patient No. : 25604072100562
PH No. : 160/24
Sex : Female
Age : 67 Y
Referring Doc :

Request No. : 13008214
Request Date : 10/01/2024
Verification Date : 08/02/2024
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, Up-Front.
Nature Specimen: Left simple mastectomy and SLNs.

Gross Examination:

Procedure: Left simple mastectomy specimen measured 15x13x2.5 cm with covering skin
Lipase measured 17x8 cm.
Localization: Retroareolar.
Tumor extent: A greyish white tumor mass.
Size: 4.5x3.5x2cm
Skin Involvement: Free.
Distance from margins: 0.1 cm away from deep margin and 0.3 cm away from the skin
Lymph nodes number and size: 5 LNs up to 2.5 cm.

Microscopic Examination:

Histological type: Invasive duct carcinoma.
Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade (2+2+1) I.
Angioinvasion: No.
Ductal carcinoma in situ: Major.
Skin Involvement: Free N/A.
Lymph node involvement: Free (0/5).
Resection Margins: Free close margin (1 mm).

Diagnosis:

Left breast, simple mastectomy and SLNs Biopsy:
Invasive duct carcinoma Grade I with major DCIS.
Free close margin (1 mm).
Free Skin.
Free (0/5).

Verified by :
DR.Menna Nabil

Printing Date : 04/03/2024 11:09:38

Page 1 of 2

Printing By : marwa mohamed youss

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / فاطمة ابراهيم محمود رزق

Request No. : 13008214

Patient No. : 25604072100562

Request Date : 10/01/2024

Path No. : 160/24

Verification Date : 08/02/2024

Sex : Female

Financial Category : نفقة الدولة - وزارة الصحة

Age : 67 Y

Department : Out Patient

Referring Doc :

Registered by : marwa mohamed youssef

Pathology Report

- Stage: pT2, pN0

Signature

Prof. Dr. Marwa Mahmoud.

Dr. Sagda Elkady.

Verified by :

DR. Menna Nabil

Printing Date : 04/03/2024 11:09:38

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Printing By : marwa mohamed youssef



شرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor - Building No. 987 - Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاد / فاطمة أبو العزم محمود رزق	Request No.	: 13000961
Patient No.	: 25604072100562	Request Date	: 03/11/2025
Path No.	: 3355/25	Verification Date	: 30/11/2025
Sex	: Female	Financial Category	: نفقة المرأة - جويستاف روسي
Age	: 69 Y	Department	: Out Patient
Refereing Doc :		Registered by	: Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right hepatic lobe focal lesion, in known case of breast cancer.
Nature of Specimen: Right hepatic lobe focal lesion, Tru-cut biopsy, collected on 02/11/2025.

Gross Examination:

- Two grey white tissue cores, each measured 2 cm long. Totally submitted.
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Right hepatic lobe focal lesion, Tru-cut biopsy:

Metastatic adenocarcinomatous deposits (Positive CK20 & CDX2), mostly consistent with colo-rectal / GIT origin.

- Case of clinical & radiological correlation.

Signature

Prof. Dr. Maha Guimei.

Verified by :

Dr.Duaa Mohamed

Printing Date : 30/11/2025 10:21:36

Page 1 of 2

Printing By : Mrs. Mayada Hegazy



RAS-BRAF Molecular Report

Name: Fatma Ibrahim Mahmoud

Age: 70 years.

Clinical data: Invasive adenocarcinoma grade 2

Specimen type: Referred Block

Date (Received): 03 / 08 / 2026

Date (reported): 06 / 08 / 2026

Lab. Number: SR-657/26

Referred by:

Indication:

Predictive test for response to therapy.

Gene	Result	Mutation Detected
KRAS	Wild type	NA
NRAS	Wild type	NA
BRAF	Wild type	NA

Interpretation & Comment

Based on the current results this patient could be considered for anti-EGFR monoclonal antibody targeted therapy. as the current tumor is expected to achieve clinical response to anti-EGFR monoclonal antibody targeted therapy.

Test Background

Approximately 40% of patients with mCRC harbor an activating KRAS & NRAS mutations in 4% of the patients. The outcome of patients with RAS mutations is worse compared to RAS wild type (RAS wt) cases. However, KRAS & NRAS mutations also have distinct impacts on survival in mCRC; the prognosis of the aggressive NRAS mutant molecular subset is poor.

According to National Comprehensive Cancer Network guidelines (NCCN; Version 1.2020) for metastatic colorectal cancer (mCRC), expanded RAS panel for activating mutation of KRAS & NRAS is crucial at time of diagnosis to select the patients who are likely to benefit from anti-EGFR therapies.

Test Description

Tissue sections were reviewed by a pathologist and relevant tumor was selected for analysis.

- DNA was isolated and amplified by quantitative polymerase chain reaction (qPCR) using NRAS and KRAS CE-IVD kits according to manufacture protocol. results interpreted according to the manufacture user guide. Mutation nomenclature was performed considering the following reference sequences: NM_004985.4 (KRAS) and NM_002524.4 (NRAS gene).
- BRAF V600E mutation specific Competitive allele specific TaqMan assay along with its reference wildtype assay were performed for each sample to accurately interpret the results.



Disclaimer:

- This test reported herein was performed at an external reference lab (57357 laboratory), verified by Biotechnology Molecular Interpretation Clinico-pathologic Interpretation
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Microsatellite Instability Testing Report

Name: Fatma Ibrahim Mahmoud

Age: 70 years.

Clinical data: Invasive adenocarcinoma grade 2

Specimen type: Referred Block

Date (Received): 04/ 08 / 2026

Date (reported): 11 / 08 / 2026

Lab. Number: SR-657/26

Referred by:

Indication:

Predictive test for response to therapy.

Result

Microsatellite Stable (MSS)

Interpretation & Comment

In the absence of microsatellite instability, are unlikely to obtain clinical benefit from Immunotherapy check points inhibitors regimens.

Test Background

MSI screening test looks for changes in the DNA sequence between normal tissue and tumor tissue and can identify whether or not there is high amount of instability, which is called MSI-High. Result could be MSI high, low or stable, MSI stable are likely to benefit from standard chemotherapeutic regimens. But MS-LOW or MS-HIGH cases are not expected to obtain clinical benefit from treatment with standard chemotherapeutic regimens. Many patients with MSI-H tumors have had a positive response to immunotherapy treatments (or immune-checkpoint therapies).

Test Description

Method: FFPE DNA Extraction: DNA was isolated from neoplastic tumor cells

- Microsatellite Analysis: A total of 5 polymorphic markers are used in the interpretation of three levels of MSI, stable (MSS), Low (MS-LOW) or High (MS-High). The markers used in this analysis included: BAT25, BAT26, NR21, NR24 and NR 27. Two control markers are also included for quality control purposes.

- Bioinformatics analysis: Resulted files were subjected to analysis pipeline for MSI stability detection using True mark MSI analysis software, ThermoFisher.

- Method Limitation: In the absence of the patient specific control sample (Normal Tissue or Blood). This method able to detect microsatellite instability only in sample at allele frequency not less than 20% by detecting the emergence of any novel allele with a different repeat length compared to the reference sequence.





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